Version:1

Bwrdd Iechyd Prifysgol
Betsi Cadwaladr
University Health Board

ANAESTHESIA, CRITICAL CARE & PAIN MANAGEMENT CPG

OBSTETRIC ANAESTHESIA WRITTEN CONTROL DOCUMENT

ANTACID PROPHYLAXIS FOR LABOUR AND DELIVERY

Date to be reviewed:	1st October 2021	**No of pages:**	3
Author(s):	Dr Gareth Roberts	**Author(s) title:**	Consultant Anaesthetist
Responsible dept / director:	Department of Anaesthesia, BCUHB (West) Dr Linda Warnock Clinical Lead of Anaesthesia, BCUHB (West)		
Approved by:	Department of Anaesthesia, BCUHB (West)		
Date approved:			
Endorsement by:	Department of Anaesthesia, BCUHB (West)		
Date endorsed:			
Date activated (live):	2nd October 2018		

Date EQIA completed:	
Documents to be read alongside this policy:	None
Review	**Purpose of Issue/Description of current changes:** New obstetric anaesthesia written control document for Betsi Cadwaladr University Health Board (West)

First operational:	January 2013				
Previously reviewed:	Date: 30/1/13	Date 1/10/18	date	date	date
Changes made yes/no:	Yes	No	Yes/no	Yes/no	Yes/no

PROPRIETARY INFORMATION
This document contains proprietary information belonging to the Betsi Cadwaladr University Health Board. Do not produce all or any part of this document without written permission from the BCU

Contents

1. Introduction and purpose ……3
2. Scope……..……3
3. Points to consider……...3
4. References……..3

Paper copies of this document should be kept to a minimum and checks made with the electronic version to ensure the version to hand is the most recent.

1. Introduction and Purpose

Gastric emptying is reduced during pregnancy and especially during labour. In pregnancy, lower oesophageal sphincter pressure is reduced and intragastric pressure is increased resulting in a reduction in barrier pressure. The purpose of antacid prophylaxis in labour and prior to Caesarean Section is to reduce the risk of aspiration pneumonitis (Mendelson's Syndrome) among women undergoing Caesarean Section under general anaesthesia. Aspiration pneumonitis carries a high risk of mortality. It's prevention involves the use of the H2 receptor antagonist ranitidine, which inhibits gastric acid production. It also involves the use of sodium citrate which acts as a buffer to neutralise any acid in the stomach.

2. Scope

This guideline is intended as reference for trainee anaesthetists, obstetricians and midwifes seeking a recommended method of antacid prophylaxis for labour and delivery.

3. Points to consider

3.1 Labouring women

Ranitidine 150mg 12 hourly orally should be given throughout labour. If the patient needs an emergency Caesarean Section, then **30mls of 0.3M Sodium Citrate orally** should be given as the patient is about to leave the labour room for theatre.

3.2 Elective Caesarean Sections

Women booked for an elective Caesarean Section should receive two doses of **ranitidine 150mg orally** as a take home prescription following preoperative assessment. One should be taken at **22:00 on the evening prior to surgery**, and the second dose at **07:00 (morning list) or 12:00 (afternoon list) on the day of surgery**. Then prior to surgery, **30 mls of 0.3M sodium citrate** should be given orally as the patient is leaving the ward for theatre.

4. References

1. Gillett GB, Watson JD, Langford RM. Ranitidine and single-dose antacid therapy as prophylaxis against acid aspiration syndrome in obstetric practice. *Anaesthesia* 1984; 39: 638-644